

D18 — Nephrology Reconsult

Dr. P. Gupta, Nephrology | Nephrology | 02/25/2026 11:30

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

NEPHROLOGY RECONSULT NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156

Date: 02/25/2026 11:30 | Provider: Dr. P. Gupta, Nephrology

Reason for Reconsult: AKI management, medication restart guidance

CURRENT STATUS:

AKI on CKD 3b. Cr peaked at 2.3 (Day 6), now 1.9. Baseline Cr 1.5. NOT yet at baseline. K+ 4.6.

RECOMMENDATIONS:

1. HOLD lisinopril. ACEi will further reduce GFR in the setting of AKI that has not yet resolved. Cr 1.9 is still significantly above baseline of 1.5. Risk of worsening renal function and hyperkalemia. Do not restart until Cr returns to ≤ 1.6 .
2. HOLD spironolactone. K+ 4.6 with ongoing AKI and impaired potassium excretion. High risk of dangerous hyperkalemia, especially if combined with ACEi restart. Do not restart until K+ consistently < 4.5 AND Cr at or near baseline.
3. Furosemide: 20 mg daily is acceptable if volume status warrants. Monitor Cr closely – diuresis could worsen AKI.
4. Discontinue KCl supplementation. K+ 4.6 with AKI – supplementation is unnecessary and dangerous. Patient will be restarting potassium-retaining medications (ACEi, potentially spironolactone) as an outpatient.
5. Metformin: continue to hold. eGFR currently ~ 35 . Contraindicated.
6. Follow-up: Outpatient nephrology in 2-3 weeks with BMP.

I note that our recommendations regarding lisinopril and spironolactone differ from cardiology's. We understand the mortality benefit of GDMT in HFrEF, but believe the acute renal risk outweighs these benefits until renal function returns closer to baseline. We defer to the medicine attending for the final decision.

Electronically signed by: Dr. P. Gupta, MD

Date: 02/25/2026 12:00

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